THE MATERNITY METHOD

NCLEX CHEAT SHEET

# Fetal Monitoring

**Maternal & Newborn** › Intrapartum

**The Big Picture** The fetal heart rate (FHR) strip shows **how well baby is getting oxygen**. Read it the same way every time: **baseline → variability → accels → decels → contractions → category**. **Moderate variability** is the most reassuring finding on the strip.

## KNOW THE NUMBERS

| 110–160 | 6–25 | 15 × 15 | > 5 in 10 min | 2 in 20 min |
|---|---|---|---|---|
| Normal **baseline** (bpm) | **Moderate** variability | **Accel**: ↑15 bpm for 15 sec (≥ 32 wks) | **Tachysystole**: too many contractions | Accels for a **reactive NST** |

## Decelerations Memory hack: **VEAL CHOP**

| Decel | Looks like | Cause | What to do |
|---|---|---|---|
| **Variable** | **Abrupt** drop, V/U/W shape, any time | **C**ord compression | **Reposition**; stop oxytocin; amnioinfusion if ordered; check for **cord prolapse** |
| **Early** | **Gradual**, mirrors the contraction | **H**ead compression | **Nothing**: it's benign. Keep watching |
| **Accel** | Brief **rise** above baseline | **O**K! | Reassuring: baby is well oxygenated |
| **Late** | Gradual, low point **after** the contraction peak | **P**lacental insufficiency | **Intrauterine resuscitation** now; notify provider |

MEMORY HACK · INTRAUTERINE RESUSCITATION

### Worrying strip? Stop the Pit, then LION

| Stop | L | I | O | N |
|---|---|---|---|---|
| **oxytocin** first | **Left** side / reposition | **IV** fluid bolus | **Oxygen** if ordered / per protocol | **Notify** provider and document |

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## Variability

- **Absent:** flat line, undetectable → ominous
- **Minimal:** ≤ 5 bpm → sleep, opioids, mag, or hypoxia
- **Moderate: 6–25 bpm → well oxygenated**
- **Marked:** > 25 bpm → evaluate

A **sleep cycle** can cause minimal variability for up to 40 min. Try gentle stimulation.

## Baseline Problems

- **Tachycardia > 160:** maternal **fever**/infection, early hypoxia, terbutaline, dehydration, hyperthyroid
- **Bradycardia < 110:** hypoxia, **cord prolapse**, maternal hypotension (epidural), late hypoxia
- **Sinusoidal** (smooth wavy pattern): severe **fetal anemia**, Category III

## FHR Categories

| Category | Includes | Action |
|---|---|---|
| I **Normal** | Baseline 110–160, **moderate** variability, no lates or variables (earlies/accels OK) | Routine monitoring |
| II **Indeterminate** | Anything not I or III (minimal variability, tachycardia, recurrent variables with moderate variability…) | **Evaluate**, resuscitate, keep watching |
| III **Abnormal** | **Absent** variability + recurrent lates, recurrent variables, or bradycardia; or a **sinusoidal** pattern | **Resuscitate** + prepare for **fast delivery** |

## External vs. Internal

- **Tocotransducer** (external): shows **frequency + duration**, **not** strength
- **IUPC** (internal): measures true **strength** in mmHg
- **FSE** (internal): needs **ruptured membranes** and a dilated cervix; not on the face
- Place the US transducer over the **fetal back** (use **Leopold** maneuvers)

## Contractions

- **Frequency:** start of one → start of the next
- **Duration:** start → end of one
- **Resting tone:** uterus should **relax** between contractions
- **Tachysystole:** > 5 in 10 min (averaged over 30 min) → **stop oxytocin**

## Antepartum Tests

- **NST: reactive (good)** = 2 or more accels in 20 min; nonreactive → more testing
- **Biophysical profile (BPP):** NST + breathing + movement + tone + fluid, 2 pts each. **8–10 = reassuring**
- **Contraction stress test: negative (no lates) = good**

### ☆ MUST-KNOW FOR THE NCLEX

- **VEAL CHOP:** Variable = Cord · Early = Head · Accel = OK · Late = Placenta.
- **Late decels or absent variability** → stop oxytocin, left side, IV bolus, $O_2$ per protocol, notify.
- **Variable decels right after membranes rupture** → check for **cord prolapse**.
- **Moderate variability** is the best sign of a well-oxygenated baby.
- Reactive NST and **negative** CST are the **good** results.

### QUICK CHECK · TEST YOURSELF

1. Normal FHR baseline is ____–____ bpm.
2. Late decelerations are caused by _____.
3. Early decelerations need _____ intervention.
4. First action for tachysystole on oxytocin: _____.
5. A smooth, wavy sinusoidal pattern suggests severe fetal _____.

**Answers:** 1. 110–160 · 2. placental insufficiency (uteroplacental insufficiency) · 3. no · 4. stop the oxytocin · 5. anemia

For study use only. Always follow facility protocols, current guidelines, and provider orders.